

Dear b.well Hiring Team,

Healthcare's fragmentation problem is a "data problem": reconciling heterogeneous sources into one trustworthy record. That is the EXACT work I have done for the last decade. A FHIR-native platform connecting data is exactly where I want to go with my career.

As Lead Data Engineer at Pharmacists Mutual, I own the architecture, strategy, and team for enterprise analytics. I rebuilt our reporting platform from scratch. I redesigned ingestion to flow through Ataccama master data management (MDM) rather than straight from source. I cut storage 50%, processing time over 60%. Integrating messy, single-owner, undocumented systems is my métier. I also recruited and onboarded a senior engineer and converted tribal knowledge into shared standards, which is the mentorship and review your posting mentioned.

On the personal side, I build with your teams same modern tools. My Gutenberg-Fingerprint project is a nightly change-data-capture pipeline running solo on a \$5/month VPS: Postgres, plain Python, and dbt Core orchestrated by Dagster, compiling against DuckDB in dev and Postgres in prod. I'm comfortable in Python and pandas, event-driven and batch patterns, containerized deployment, and IaC with Terraform. I would ramp quickly on FHIR controls, having already worked inside a regulated, compliance-heavy insurer.

The AI dimension is what makes b.well compelling to me, now, in this moment. I championed Claude Code adoption across my org and authored the security framework that cleared it for production. Seeing OpenAI select b.well as the health data layer for ChatGPT... I want to help build that secure, well-modeled foundation.

Best regards,
Sander VanWilligen